



DHIS2 PREVENTION DATABASE ANNUAL MONITORING REPORT 2025

Strategic Information Component

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INTRODUCTION

The District Information System (DHIS2) database provides an overview of individuals accessing HIV prevention services. The DHIS2 database has been under the direct management of the HSTU/MOHW since August 2024. The system aims to facilitate the collection, management, and analysis of data critical for monitoring service delivery, assessing outreach effectiveness, and informing programme decisions. HIV prevention services encompass various interventions such as, reach (educational interaction), HIV counselling and testing, condom distribution, and pre-exposure prophylaxis (PrEP). Tracking the uptake and outcomes of these services helps the programme to assess coverage, identify gaps, and tailor interventions to populations at higher risk.

DATABASE OVERVIEW

The DHIS2 is designed to store information sourced from the outreach register utilized in the field. In this system, each record corresponds to a specific event or service, organized by location, date, and type of event. The structure includes event records that detail the service provided, as well as when, where, to whom, and by whom it was delivered. Each event is connected to a scheduled outreach programme activity, establishing a direct link between the database and the on-the-ground activities.

The outreach register is the primary source of information entered into the DHIS2 system, ensuring that the data accurately reflects the real activities and interactions occurring during outreach efforts. Each entry denotes a unique event or service encounter, characterized by its location, date, and type. Key details captured for each event include:

Location: The venue where the event took place.

Date: The date on which the event occurred.

Population Category: Groups served, including Men Who Have Sex with Men (MSM), Transgender Women (TG), Female Sex Workers (FSW), At-Risk Youth (ARY), Sexually Active Males (SAM), Sexually Active Females (SAF), Males aged fifty and older (M50+), and Females aged fifty and older (F50+).

Type of Event: The nature of the service or interaction, such as HIV counselling and testing, condom distribution, or risk assessment.

This "event-based" structure is particularly effective for monitoring service provision and tracking the progress of outreach programmes.

DHIS2 MAINTENANCE

The DHIS2 database undergoes routine backups to ensure data integrity and security. Automatic daily backups are conducted, complemented by manual backups performed weekly by the Database Manager. Additionally, the entire system is backed up quarterly and securely stored on an external hard drive, maintained by the Information and Communication Technology Unit (ICTU). During the update period from July to September 2025, the following changes were implemented:

1. The psychosocial services field has been renamed to social services to better reflect referrals to other health services.
2. A differentiation field for the type of sex that clients engaged in has been added to capture high-risk behaviours.
3. New fields have been introduced to record instances of gender-based violence, mental health, and the number of multiple-sex partners.

CHALLENGES AND LIMITATIONS

The DHIS2 does not prevent duplicated entries; hence, there is a manual process for de-duplication. This is a critical limitation. Ideally, a database should have built-in mechanisms to prevent the creation of duplicate records. The fact that DHIS2 doesn't prevent duplicates means that the data is susceptible

to inflation and inaccuracies. A manual de-duplication process is time-consuming and It requires staff to use excel and review records and identify potential duplicates.

DHIS2 is not able to track individuals, whether it is within a site or across sites, because each entry is seen as an event. This is a fundamental design constraint of this particular DHIS2 implementation. Because the system treats each service encounter as an independent "event," it is difficult to track an individual's journey across multiple services or sites over time. This makes it challenging to create a longitudinal view of a client's engagement with the programme.

OVERVIEW OF HIV PREVENTION SERVICE DELIVERY (2025)

Overall, the programme demonstrates a broad and impactful approach, focusing on targeted and key populations at high risk for HIV/STIs acquisition. Prevention services combines education, counselling, testing, and referrals to promote safer behaviours and improve health outcomes.

A catastrophic category five hurricane struck the island in October 2025, causing damage to health facilities, power outage and interruption in internet connectivity. This affected real-time data entry and access to DHIS2, especially among the most affected parishes in the Western Regional Health Authority (WRHA) and Southern Regional Health Authority (SRHA). Displacement of outreach officers and patrons in the affected parishes further compounded the routine updating of the database and health-seeking behaviours which may have impacted Prevention activities during the period of October to December 2025.

For the reporting period, the HIV Programme interventions successfully reached a total of 152,680 individuals across targeted populations, with the highest engagement seen in the sexually active females (SAF), followed by sexually active males (SAM). Key populations, female sex workers (FSW), men who have sex with men (MSM) and Transgender (TG), contributed 11,421, 5,781 and 482, respectively, accounting for 12% of the overall reach total. All individuals participated in risk reduction conversations, which served as a major strategy for education and prevention, reaching all groups extensively.

Persons identified with high-risk behaviours, occurred in 9,447 instances highlighting ongoing challenges in risk engagement among these populations. Referrals to additional health services were made in 3,593 instances, facilitating access to comprehensive care.

HIV screening efforts were also significant, with 136,314 people tested, accounting for 89% of the total number reached. The SAF population again had the highest testing numbers, followed by At-Risk-Youth (ARY) and SAM. Distribution of HIV self-test kits was 7,234 across all populations; the lowest distribution continues to be among MSM and TG.

A total of 899,019 pieces of condoms were distributed, with the highest distribution among SAF and SAM populations, indicating a focus on prevention tools. Pre-exposure prophylaxis (PrEP) was provided to 609 individuals, mainly within the broader at-risk populations.

Table 1: Total Entries in DHIS2 by Service Delivery and Populations January to December 2025

Intervention	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Total
Risk Reduction Conversation	34,032	9,891	8,708	50,722	31,643	11,421	5,781	482	152,680
High Risk Behaviour identified*	1,837	259	370	1,515	2,145	2,523	731	67	9,447
Reach	34,032	9,891	8,708	50,722	31,643	11,421	5,781	482	152,680
Testing	30,654	8,794	7,414	46,175	26,689	10,713	5,454	451	136,314
HIV/ST	1,379	348	394	2,496	2,357	172	82	6	7234
Condom Distribution	175,999	41,772	41,465	274,679	172,282	158,906	29,740	4,176	899,019
Referrals of services	435	380	240	608	540	527	802	61	3,593
PrEP	60	3	0	11	169	108	250	8	609

*a person is considered “high risk” if they seek treatment for STIs, substance misuse or both

DATA ANALYSIS

During the year, all reports were submitted by implementing entities ((South East Regional Health Authority, Western Regional Health Authority, North East Regional Health Authority, Southern Regional Health Authority, Children of Faith, Children First Agency, ASHE Company Limited and Jamaica AIDS Support for Life) to facilitate reporting and data validation. No backlog was reported by any of the entities. (Table 2).

Table 2: Reports Validation

Expected Reports	Received Reports	%
84	84	100%

RISK REDUCTION CONVERSATIONS

For the reporting period, 151,827 conversations were had with clients around inconsistent or no condom and lubricant use. Discussions were held mainly with sexually active females, ARY and SAM (50,509, 33,867 and 31,329 respectively). Table 3 contains a detailed breakdown.

Table 3: Number of Persons Engaged: Condom and Lube Usage by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	50	0	2	2	6	0	471	5	536
Children First	2	3	0	3	4	738	621	27	1,398
Children of Faith	40	18	17	93	56	40	4	0	268
JASL	904	125	147	1,179	674	1,839	1,808	28	6,704
NERHA	4,288	2,183	1,805	7,824	5,792	1,384	301	60	23,637
SERHA	13,395	2,298	2,540	17,367	11,317	2,022	320	130	49,389
SRHA	6,188	2,406	1,769	10,429	5,676	1,517	855	31	28,871
WRHA	9,000	2,788	2,368	13,612	7,804	3,860	1,391	201	41,024
Grand Total	33,867	9,821	8,648	50,509	31,329	11,400	5,771	482	151,827

Stigma and discrimination risk reduction conversations were had with 57,712 clients over the reporting period. SAF accounted for the most (16,846) followed by ARY (10,986) and SAM (9,866). Table 4 contains a detailed breakdown.

Table 4: Number of Persons Engaged: Stigma and Discrimination by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	280	2	282
Children First	2	3	0	3	4	730	458	20	1,220
Children of Faith	0	0	0	2	0	0	0	0	2
JASL	161	27	24	201	116	434	700	10	1,673
NERHA	143	84	97	230	161	74	20	2	811
SERHA	689	135	200	619	590	152	39	5	2,429
SRHA	5,684	2,258	1,679	9,685	5,357	1,433	768	30	26,894
WRHA	4,307	1,281	1,146	6,106	3,638	1,987	809	127	19,401
Grand Total	10,986	3,788	3,146	16,846	9,866	4,810	3,074	196	52,712

Partner reduction conversations were had with 32,434 clients who reported having multiple partners for the reported period. SAF accounted for the most (9,527) followed by ARY (7,297) and SAM (6,785). The entity with the most report was SRHA which recorded 12,830. Table 5 contains a detailed breakdown.

Table 5: Number of Persons Engaged: Partner Reduction by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	15	0	1	0	2	0	139	0	157
Children First	1	0	0	0	0	5	274	17	297
Children of Faith	28	10	11	60	38	27	0	0	174
JASL	273	38	37	281	129	162	265	2	1,187
NERHA	292	94	85	522	358	105	40	10	1,506
SERHA	1,837	188	304	1,434	1,936	864	58	17	6,638
SRHA	2,931	869	663	4,643	2,488	742	478	16	12,830
WRHA	1,920	596	543	2,587	1,834	1,641	456	68	9,645
Grand Total	7,297	1,795	1,644	9,527	6,785	3,546	1,710	130	32,434

A total of 18,899 risk reduction conversations were had with clients seeking care for sexually transmitted infections (STIs) for the reporting period. SAF accounted for the most (6,690) followed by ARY (4,279) and SAM (2,315). The entity with the most report was WRHA which recorded 9,985. Table 6 contains a detailed breakdown.

Table 6: Number of Persons Engaged: Seeking Care for STIs by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	14	0	14
Children First	1	0	0	0	0	3	177	7	188
JASL	3	3	1	3	1	4	6	0	21
NERHA	32	22	17	32	35	13	7	0	158
SERHA	18	5	7	31	22	13	4	3	103
SRHA	680	267	202	1,002	540	157	124	3	2,975
WRHA	3,545	955	782	5,622	2,961	1,212	326	37	15,440
Grand Total	4,279	1,252	1,009	6,690	3,559	1,402	658	50	18,899

A total of 12,925 conversations around violence reduction were conducted. Sexually Active Female population accounted for the most (4,128) followed by ARY (2,527) and SAM (2,024). The entity with the most reports was NERHA, which recorded 10,612. Table 7 contains a detailed breakdown.

Table 7: Number of Persons Engaged: Violence Reduction by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	23	2	25
Children First	0	0	0	0	0	0	11	1	12
JASL	101	4	10	122	44	275	95	2	653
NERHA	2,051	1,595	1,018	3,450	1,684	716	77	21	10,612
SERHA	30	10	7	47	26	29	2	0	151
SRHA	45	10	9	61	38	16	24	1	204
WRHA	300	73	58	448	232	100	42	15	1,268
Grand Total	2,527	1,692	1,102	4,128	2,024	1,136	274	42	12,925

A total of 13,483 conversations around substance abuse were had. SAF accounted for the largest proportion (4,844) followed by ARY (3,270) and SAM (2,422). The entity with the most reports was WRHA, which recorded 9,295. Table 8 contains a detailed breakdown.

Table 8: Number of Persons Engaged: Substance Abuse by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	4	0	0	0	0	0	73	1	78
Children First	0	0	0	0	0	0	16	2	18
Children of Faith	1	0	0	1	1	4	0	0	7
JASL	5	0	0	2	5	10	3	0	25
NERHA	19	3	1	13	5	0	1	0	42
SERHA	15	0	2	12	12	21	0	0	62
SRHA	1,008	364	249	1,313	643	180	190	9	3,956
WRHA	2,218	571	434	3,503	1,756	633	159	21	9,295
Grand Total	3,270	938	686	4,844	2,422	848	442	33	13,483

A total of 9,281 risk reduction conversations were had around anal and genital care. SAF accounted for the largest proportion (3,436) followed by ARY (2,090) and SAM (1,422). The entities with the most reports were WRHA and SRHA, which recorded 4,603 and 4,233, respectively. Table 9 contains a detailed breakdown.

Table 9: Number of Persons Engaged: Anal/Genital Care by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	8	0	8
Children First	1	0	0	0	0	0	128	9	138
JASL	0	1	0	1	1	0	2	0	5
NERHA	55	11	7	48	32	7	7	0	167
SERHA	20	2	7	20	15	30	25	8	127
SRHA	941	310	152	1,780	565	253	229	3	4,233
WRHA	1,073	271	212	1,587	809	295	321	35	4,603
Grand Total	2,090	595	378	3,436	1,422	585	720	55	9,281

A total of 13,262 conversations around abstinence were had. SAF accounted for the largest proportion (4,343), followed by ARY (3,159) and SAM (2,540). The entity with the most reports was SRHA, which recorded 7,642. Table 10 contains a detailed breakdown.

Table 10: Number of Persons Engaged: Abstinence by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	42	0	2	2	5	0	176	0	227
Children First	1	0	0	0	0	0	69	8	78
Children of Faith	40	18	17	92	54	40	4	0	265
JASL	8	1	0	10	11	4	1	0	35
NERHA	104	6	1	8	2	0	4	1	126
SERHA	218	17	17	114	55	35	7	0	463
SRHA	1,692	630	524	2,693	1,543	370	183	7	7,642
WRHA	1,054	298	272	1,424	870	337	157	14	4,426
Grand Total	3,159	970	833	4,343	2,540	786	601	30	13,262

A total of 151,914 conversations around basic facts about HIV were had. SAF accounted for the largest proportion (50,515), followed by ARY (33,898) and SAM (31,327). The entities with the most reports were SERHA, WRHA and SRHA which recorded 49,370, 41,107 and 28,859, respectively. Table 11 contains a detailed breakdown.

Table 11: Number of Person Engaged: Basic Facts about HIV by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	50	0	2	2	6	0	472	5	537
Children First	2	3	0	3	4	738	620	27	1,397
Children of Faith	40	18	17	93	56	40	4	0	268
JASL	904	124	147	1,176	671	1,836	1,808	28	6,694
NERHA	4,302	2,221	1,808	7,819	5,789	1,382	301	60	23,682
SERHA	13,394	2,298	2,536	17,365	11,305	2,021	321	130	49,370
SRHA	6,186	2,406	1,767	10,424	5,673	1,517	855	31	28,859
WRHA	9,020	2,793	2,375	13,633	7,823	3,867	1,395	201	41,107
Grand Total	33,898	9,863	8,652	50,515	31,327	11,401	5,776	482	151,914

A total of 16,478 conversations around family planning were had. SAF accounted for the largest proportion (6,914), followed by ARY (4,260) and SAM (1,756). The entity with the most reports was WRHA, which recorded 6,981. Table 12 contains a detailed breakdown.

Table 12: Number of Persons Engaged: Family Planning by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
Children First	0	0	0	0	0	1	2	0	3
Children of Faith	2	0	0	0	0	0	0	0	2
JASL	0	0	0	1	0	0	0	0	1
NERHA	729	217	45	1,662	48	274	3	4	2,982
SERHA	238	11	5	261	53	35	2	1	606
SRHA	1,624	377	216	2,332	797	347	203	7	5,903
WRHA	1,667	391	230	2,658	858	982	154	41	6,981
Grand Total	4,260	996	496	6,914	1,756	1,639	364	53	16,478

HIGH RISK BEHAVIOURS IDENTIFIED

During risk reduction conversations, high-risk behaviours of clients were identified. For the reported period, 2,430 clients presented with a history of sexually transmitted infections (STI). SAF and ARY account for the majority (753 and 611 respectively).

Table 13: High Risk Behaviour Identified: History of Sexually Transmitted Infections by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	1	0	1
Children First	1	1	0	0	0	8	14	2	26
Children of Faith	1	0	0	0	0	1	0	0	2
JASL	6	1	2	4	2	4	11	0	30
NERHA	105	68	44	129	50	32	4	2	434
SERHA	202	34	31	248	166	22	7	2	712
SRHA	62	48	15	107	36	25	18	0	311
WRHA	234	35	33	265	191	106	42	8	914
Grand Total	611	187	125	753	445	198	97	14	2,430

Persons identified as having inconsistent condom use totalled 43,825, for the reported period. SAF accounted for the majority (14,911), followed by ARY (9,971) and SAM (8,596). The entities with the most reports were SERHA and WRHA with 16125 and 14,141 respectively. Table 14 contains a detailed breakdown.

Table 14: High Risk Behaviour Identified: Incorrect Condom Use by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	13	0	1	1	3	0	84	0	102
Children First	1	3	0	3	4	545	109	4	669
Children of Faith	1	2	1	0	3	4	0	0	11
JASL	287	31	56	383	231	957	493	7	2,445
NERHA	1,068	754	626	1,774	1,091	455	49	14	5,831
SERHA	4,341	683	664	6,068	3,629	577	120	43	16,125
SRHA	1,046	267	204	1,759	838	207	177	3	4,501
WRHA	3,214	804	668	4,923	2,797	1,240	425	70	14,141
Grand Total	9,971	2,544	2,220	14,911	8,596	3,985	1,457	141	43,825

Individuals indicating that they had multiple sex partners totalled 9,495 for the reporting period. Female sex workers accounted for the majority (3,383), followed by SAM (2,213) and ARY (1,658). The entities with the most reports were WRHA and SERHA, having 2,994 and 2,851, respectively. Table 15 contains a detailed breakdown.

Table 15: High Risk Behaviour Identified: Multiple Sex Partners by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	14	0	1	0	2	0	133	0	150
Children First	1	0	0	1	0	545	96	4	647
Children of Faith	1	1	0	0	0	3	0	0	5
JASL	47	4	11	68	62	165	118	0	475
NERHA	199	53	93	126	227	546	28	7	1,279
SERHA	742	27	117	362	921	634	37	11	2,851
SRHA	169	28	42	169	278	313	95	0	1,094
WRHA	485	30	96	218	723	1,177	227	38	2,994
Grand Total	1,658	143	360	944	2,213	3,383	734	60	9,495

A total of 2,553 individuals indicated that they misused substance for the reporting period. SAF accounted for the majority (755), followed by ARY (594) and SAM (518). The entity with the most reports was WRHA, having 2,260. Table 16 contains a detailed breakdown.

Table 16: High Risk Behaviour Identified: Substance Misuse by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	2	0	0	0	0	0	13	0	15
Children First	0	0	0	0	0	21	3	0	24
JASL	9	0	3	17	17	32	22	0	100
NERHA	5	2	2	2	9	45	0	0	65
SERHA	2	1	0	2	8	4	1	0	18
SRHA	17	0	1	14	15	9	15	0	71
WRHA	559	91	104	720	469	232	63	22	2,260
Grand Total	594	94	110	755	518	343	117	22	2,553

A total of 979 individuals reported engaging in anal sex and orgies for the reporting period. Men Who Have Sex with Men accounted for the majority (691), followed by FSW (90) and SAF (61). The entity with the most reports was WRHA, having 319, followed by Children First, having 221. Table 17 contains a detailed breakdown.

Table 17: High Risk Behaviour Identified: Type of Sex Act (Anal/Orgies) by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	184	1	185
Children First	0	0	0	0	0	3	212	6	221
JASL	4	0	2	1	2	0	2	0	11
NERHA	18	14	4	21	4	44	13	3	121
SERHA	1	0	1	2	1	1	13	2	21
SRHA	3	0	0	2	2	12	82	0	101
WRHA	19	4	7	35	16	30	185	23	319
Grand Total	45	18	14	61	25	90	691	35	979

HIV TESTING AND MODALITIES

Site-based service delivery is conducted at bar, correctional facility, exotic club, health centres, hospitals, hotels, Motel, guest houses, indoor hangouts, lockups, Massage parlours, Non-Government Organizations (CSO), Civil Society Organizations (CSO), Faith-based Organizations (FBO), nightclubs, outside hangouts and workplaces. This modality was used to reach a total of 113,704 members of the vulnerable populations. Of those, 100,768 were tested, with 415 positives identified. This modality was mainly used by SERHA (43,373/38/848) and WRHA (29,121/26,924). Table 18 contains a breakdown of site-based modality.

Table 18: Site-based HIV Testing by Entity and Targeted Populations

Service Delivery Modality: SITE										
Target Population		ASHE	CF	COF	JASL	NERHA	SERHA	SRHA	WRHA	Total
FSW	Reached	0	702	41	795	1,214	1,831	1,267	3,153	9,003
	Tested	0	678	38	779	902	1,728	1,247	2,999	8,371
	Positive	0	7	0	3	5	11	11	10	47
MSM	Reached	470	384	4	1,206	221	259	579	1,067	4,190
	Tested	469	358	4	1,136	178	244	570	1,027	3,986
	Positive	9	6	0	28	3	5	6	14	71
TG	Reached	5	21	0	16	32	108	17	153	352
	Tested	5	15	0	14	27	100	17	141	319
	Positive	1	1	0	1	0	6	1	0	10
ARY	Reached	50	1	40	415	2,748	11,739	3,910	5,938	24,841
	Tested	50	1	39	411	2,131	10,429	3,719	5,538	22,318
	Positive	0	0	1	2	2	20	8	11	44
SAF	Reached	2	3	93	633	5,702	15,264	6,884	9,442	38,023
	Tested	2	3	91	621	4,141	14,157	6,583	8,793	34,391
	Positive	0	0	0	8	10	40	23	30	111
SAM	Reached	6	4	56	462	4,548	10,002	3,955	5,736	24,769
	Tested	6	4	55	459	2,867	8,557	3,503	5,226	20,677
	Positive	0	0	0	4	4	20	14	23	65
F50+	Reached	0	3	18	65	1,344	1,934	1,094	1,967	6,425
	Tested	0	3	18	61	1,052	1,761	1,047	1,718	5,660
	Positive	0	0	0	1	4	14	4	11	34
M50+	Reached	2	0	17	86	1,283	2,236	812	1,665	6,101
	Tested	2	0	17	83	838	1,872	752	1,482	5,046
	Positive	0	0	0	3	3	13	2	12	33
Total	Reached	535	1,118	269	3,678	17,092	43,373	18,518	29,121	113,704
	Tested	534	1,062	262	3,564	12,136	38,848	17,438	26,924	100,768
	Positive	10	14	1	50	31	129	69	111	415

Venue-based outreach activities are conducted both indoor and outdoor. This modality was used to reach a total targeted population of 36,007 members of the various populations. Of those, 33,141 were tested, with 96 positives identified. This modality was mainly used by WRHA (11,382/10,887) and SRHA (10,361/10,135). Table 19 contains a breakdown of the venue-based modality.

Table 19: Venue-based HIV Testing by Entity and Targeted Populations

Service Delivery Modality: VENUE									
Target Population		ASHE	CF	JASL	NERHA	SERHA	SRHA	WRHA	Total
FSW	Reached	0	36	1,039	169	141	243	671	2,299
	Tested	0	19	999	157	141	241	668	2,225
	Positive	0	0	6	0	1	1	0	8
MSM	Reached	2	62	596	79	48	262	272	1,321
	Tested	0	61	499	67	47	260	266	1,200
	Positive	0	3	4	0	3	1	5	16
TG	Reached	0	2	10	7	8	14	44	85
	Tested	0	2	10	5	8	13	44	82
	Positive	0	0	0	0	0	0	0	0
ARY	Reached	0	1	490	1,476	1,308	2,233	2,930	8,438
	Tested	0	1	490	1,049	1,235	2,196	2,766	7,737
	Positive	0	1	0	0	0	4	4	9
SAF	Reached	0	0	546	2,024	1,717	3,480	4,013	11,780
	Tested	0	0	545	1,552	1,659	3,402	3,858	11,016
	Positive	0	0	6	2	5	3	6	22
SAM	Reached	0	0	210	1,157	1,065	1,850	2,014	6,296
	Tested	0	0	210	697	1,010	1,774	1,903	5,594
	Positive	0	0	1	1	5	1	6	14
F50+	Reached	0	0	60	868	313	1,309	768	3,318
	Tested	0	0	60	630	298	1,293	735	3,016
	Positive	0	0	1	7	1	5	4	18
M50+	Reached	0	0	61	512	257	970	670	2,470
	Tested	0	0	61	363	244	956	647	2,271
	Positive	0	0	1	2	0	3	3	9
Total	Reached	2	101	3,012	6,292	4,857	10,361	11,382	36,007
	Tested	0	83	2,874	4,520	4,642	10,135	10,887	33,141
	Positive	0	4	19	12	15	18	28	96

The Mall/Plaza modality involves engaging with clients in public spaces where individuals meet new sexual partners. As shown in Table 20, this modality reached a total of 2,156 people from all populations, with 1,680 tested. Nine (9) HIV-positive cases were identified through this approach. The main entities utilising this modality were SERHA, which reached 1,078 individuals and tested 877, followed by WRHA, which reached 379 individuals and tested 372.

Table 20: Mall/Plaza HIV Testing by Entity and Targeted Population

Service Delivery Modality: MALL/PLAZA								
Target Population		CF	JASL	NERHA	SERHA	SRHA	WRHA	Total
FSW	Reached	1	0	2	35	5	35	78
	Tested	1	0	2	35	5	35	78
	Positive	0	0	0	1	0	0	1
MSM	Reached	104	5	2	10	13	27	161
	Tested	104	5	2	10	13	27	161
	Positive	3	0	0	1	0	0	4
TG	Reached	2	0	0	5	0	2	9
	Tested	2	0	0	2	0	2	6
	Positive	0	0	0	0	0	0	0
ARY	Reached	0	0	82	308	71	121	582
	Tested	0	0	22	233	66	117	438
	Positive	0	0	0	0	0	0	0
SAF	Reached	0	0	119	404	98	114	735
	Tested	0	0	34	366	93	112	605
	Positive	0	0	0	1	0	1	2
SAM	Reached	0	2	102	244	41	45	434
	Tested	0	2	22	186	36	44	290
	Positive	0	0	0	0	0	0	0
F50+	Reached	0	0	11	35	12	17	75
	Tested	0	0	2	28	10	17	57
	Positive	0	0	0	1	0	1	2
M50+	Reached	0	0	21	37	6	18	82
	Tested	0	0	5	17	5	18	45
	Positive	0	0	0	0	0	0	0
Total	Reached	107	7	339	1,078	246	379	2,156
	Tested	107	7	89	877	228	372	1,680
	Positive	3	0	0	4	0	2	9

HIV services are delivered through various modalities, one of which is the home-based approach. This modality involves conducting interactions within private residences. As shown in Table 21, the home-based modality was utilised by different entities to reach targeted populations. The total of all targeted populations reached was 753 and 670 tested. Six (6) HIV-positive individuals were identified among those tested via this modality. This modality was mainly exercised in WRHA and SERHA. The target population mainly reached and tested by this modality were SAF, ARY and SAM.

Table 21: Home-based HIV Testing by Entity and Targeted Population

Service Delivery Modality: HOME								
Target Population		CF	JASL	NERHA	SERHA	SRHA	W RHA	Total
FSW	Reached	0	3	0	17	3	15	38
	Tested	0	3	0	16	3	14	36
	Positive	0	0	0	0	0	0	0
MSM	Reached	72	2	0	4	1	28	107
	Tested	70	2	0	4	1	28	105
	Positive	2	0	0	2	0	0	4
TG	Reached	2	2	21	7	0	2	34
	Tested	2	1	0	7	0	2	12
	Positive	0	0	0	1	0	0	1
ARY	Reached	0	0	2	72	15	69	158
	Tested	0	0	2	66	15	65	148
	Positive	0	0	0	0	0	0	0
SAF	Reached	0	0	1	52	17	100	170
	Tested	0	0	1	45	17	86	149
	Positive	0	0	0	0	0	0	0
SAM	Reached	0	0	1	42	24	63	130
	Tested	0	0	1	36	24	57	118
	Positive	0	0	0	0	0	1	1
F50+	Reached	0	0	0	18	3	42	63
	Tested	0	0	0	16	3	33	52
	Positive	0	0	0	0	0	0	0
M50+	Reached	0	0	0	23	3	27	53
	Tested	0	0	0	21	3	26	50
	Positive	0	0	0	0	0	0	0
Total	Reached	74	7	25	235	66	346	753
	Tested	72	6	4	211	66	311	670
	Positive	2	0	0	3	0	1	6

Data extracted from DHIS2 is used to formulate all reports to stakeholders at the sub-national, national, and international. Table 22 displays the output from DHIS2 for the reporting period, with regards to reach and test per population. For the reporting period a total of 152,680 clients were reached and 136,314 tested with 526 positives identified (0.4% yield). Sexually active females accounted for the largest number reached and tested, with 50,722 and 46,175 respectively and a yield of 0.3%.

Table 22: Reach, Test and Positivity Yield Achievements January to December 2025 By Entity.

Entity	Reach/Testing	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Total
ASHE	Reached	50	*	*	2	472	2	6	5	537
	Tested	50	*	*	2	469	2	6	5	534
	Positive	0	*	*	0	9	0	0	1	10
	Yield	0%	*	*	0%	2%	0%	0%	20%	2%
Children First	Reached	2	3	739	*	622	3	4	27	1,400
	Tested	2	3	698	*	593	3	4	21	1,324
	Positive	1	0	7	*	14	0	0	1	23
	Yield	50%	0%	1%	*	2%	0%	0%	5%	2%
Children of Faith	Reached	40	18	41	17	4	93	56	0	269
	Tested	39	18	38	17	4	91	55	0	262
	Positive	1	0	0	0	0	0	0	0	1
	Yield	3%	0%	0%	0%	0%	0%	0%	0%	0%
JASL	Reached	905	125	1,839	147	1,809	1,179	674	28	6,706
	Tested	901	121	1,783	144	1,642	1,166	671	25	6,453
	Positive	2	2	9	4	32	14	5	1	69
	Yield	0%	2%	1%	3%	2%	1%	1%	4%	1%
NERHA	Reached	4,308	2,225	1,385	1,816	302	7,847	5,809	60	23,752
	Tested	3,204	1,686	1,061	1,206	247	5,729	3,587	32	16,752
	Positive	2	11	5	5	3	12	5	0	43
	Yield	0%	1%	0.5%	0%	1%	0.2%	0.1%	0%	0.3%
SERHA	Reached	13,438	2,304	2,024	2,553	321	17,448	11,363	130	49,581
	Tested	11,974	2,106	1,920	2,154	305	16,238	9,797	119	44,613
	Positive	20	16	13	13	11	46	25	7	151
	Yield	0.2%	1%	1%	1%	4%	0.3%	0%	6%	0.3%
SRHA	Reached	6,229	2,419	1,518	1,792	855	10,479	5,871	31	29,194
	Tested	5,996	2,354	1,496	1,717	844	10,095	5,337	30	27,869
	Positive	12	9	12	5	7	26	15	1	87
	Yield	0.2%	0.4%	1%	0.3%	1%	0.3%	0.3%	3%	0.3%
WRHA	Reached	9,060	2,797	3,875	2,381	1,396	13,671	7,860	201	41,241
	Tested	8,488	2,506	3,717	2,174	1,350	12,851	7,232	189	38,507
	Positive	15	16	10	15	19	37	30	0	142
	Yield	0%	1%	0.3%	1%	1%	0.3%	0.4%	0%	0.4%
National Total	Reached	34,032	9,891	11,421	8,708	5,781	50,722	31,643	482	152,680
	Tested	30,654	8,794	10,713	7,414	5,454	46,175	26,689	421	136,314
	Positive	53	54	56	42	95	135	80	11	526
	Yield	0.2%	0.6%	0.5%	0.6%	1.7%	0.3%	0.3%	2.6%	0.4%

HIV SELF TESTING

HIV self-testing among all populations was 7,234, which was mainly for primary use. NERHA distributed the most tests, totalling 251 for primary use and 228 for secondary use. The main populations utilizing the tests are SAF and SAM with 2,496 and 2,357 individuals respectively. For a detailed breakdown, refer to Table 23.

Table 23: HIV Self-Test Distribution Method by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
Children First	0	0	0	0	0	0	1	0	1
Primary	0	0	0	0	0	0	1	0	1
Secondary	0	0	0	0	0	0	0	0	0
Children of Faith	0	0	0	2	1	2	0	0	5
Primary	0	0	0	1	1	2	0	0	4
Secondary	0	0	0	1	0	0	0	0	1
NERHA	625	174	233	1,172	1,184	67	30	4	3,489
Primary	342	117	114	760	551	55	23	3	1,965
Secondary	283	57	119	412	633	12	7	1	1,524
SERHA	414	79	53	673	481	41	13	2	1,756
Primary	274	48	44	371	330	17	10	2	1,096
Secondary	140	31	9	302	151	24	3	0	660
SRHA	241	71	81	473	509	15	35	0	1,425
Primary	121	41	39	295	177	11	29	0	713
Secondary	120	30	42	178	332	4	6	0	712
WRHA	99	24	27	176	182	47	3	0	558
Primary	88	19	23	143	153	39	1	0	466
Secondary	11	5	4	33	29	8	2	0	92
Grand Total	1,379	348	394	2,496	2,357	172	82	6	7,234

An overview of HIV self-testing administration across different entities and populations, distinguishing between assisted and unassisted testing, as well as reactive and non-reactive results, revealed that most tests were unassisted (5,875) versus assisted (1,360). This implies that clients prefer self-administered testing. There were 4 reactive results, with 1 coming from ARY, FSW, SAF and SAM each. SAF (2,496) and SAM (2,357) have the overall highest distribution of all the populations. NERHA had the highest numbers distributed. See Table 24 for a detailed breakdown.

Table 24: HIV Self-Test Administration by Entities and Target Population

Entity	Target Population							
Children First	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	0	0	0	0	0	0	0	0
Assisted Reactive	0	0	0	0	0	0	0	0
Assisted Non-reactive	0	0	0	0	0	0	0	0
Unassisted	0	0	0	0	1	0	0	0
Unassisted Reactive	0	0	0	0	0	0	0	0
Unassisted Non-Reactive	0	0	0	0	1	0	0	0
Children of Faith	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	0	0	0	0	0	0	1	0
Assisted Reactive	0	0	0	0	0	0	0	0
Assisted Non-reactive	0	0	0	0	0	0	1	0
Unassisted	0	0	0	2	0	2	0	0
Unassisted Reactive	0	0	0	0	0	0	0	0
Unassisted Non-Reactive	0	0	0	0	0	1	0	0
NERHA	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	17	6	7	5	6	30	27	1
Assisted Reactive	0	0	0	0	0	0	1	0
Assisted Non-reactive	59	18	8	22	8	112	128	0
Unassisted	608	168	60	228	24	1,142	1,157	3
Unassisted Reactive	0	0	0	1	0	0	0	0
Unassisted Non-Reactive	30	8	23	12	9	65	76	0
SERHA	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	287	56	29	38	3	445	324	1
Assisted Reactive	0	0	0	0	0	0	0	0
Assisted Non-reactive	16	2	4	1		6	12	1
Unassisted	127	23	13	15	10	228	157	1
Unassisted Reactive	0	0	0	0	0	0	0	0
Unassisted Non-Reactive	0	1	0	0	0	0	0	0
SRHA	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	5	0	4	1	0	6	16	0
Assisted Reactive	1	0	0	0	0	0	0	0
Assisted Non-reactive	3	0	4	0	0	1	4	0
Unassisted	236	71	11	80	35	467	493	0
Unassisted Reactive	0	0	0	0	0	0	0	0
Unassisted Non-Reactive	24	2	0	7	4	51	41	0
WRHA	ARY	F50+	M50+	FSW	MSM	SAF	SAM	TG
Assisted	4	2	15	4		12	8	0
Assisted Reactive	0	0	0	0	0	1	0	0
Assisted Non-reactive	3	1	1	1	0	7	4	0
Unassisted	95	22	32	23	3	164	174	0
Unassisted Reactive	0	0	0	0	0	0	0	0
Unassisted Non-Reactive	8	1	5	6		15	10	0
Total Assisted	313	64	55	48	9	493	376	2
Total Unassisted	1,066	284	118	346	73	2,003	1,981	4
Total	1,379	348	173	394	82	2,496	2,357	6
Total Reactive	1	0	0	1	0	1	1	0
Yield	0.07%	0%	0.58%	0%	0%	0.04%	0.04%	0%

CONDOM DISTRIBUTION

Table 25 provides data on condom distribution across different entities and various targeted populations. In total 899,019 pieces of condoms were distributed, with SAF accounting for the highest (274,679) followed by ARY (175,999) while MSM (29,740) and TG (4,176) accounted for the lowest. Condom distribution documentation in DHIS2 was very low for ASHE (373), JASL (1,030) and Children of Faith (3,376). This will be addressed in prevention meetings.

Table 25: Condom Distribution by Target Population and Entities

Entity	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	0	0	0	0	373	0	0	0	373
Children First	12	36	9,276	0	854	36	60	384	10,658
Children of Faith	373	129	1,447	108	20	762	537	0	3,376
JASL	99	0	337	0	555	12	27	0	1,030
NERHA	7,348	4,743	62,372	5,996	1,345	18,164	17,588	513	118,069
SERHA	61,956	9,830	20,297	11,695	2,158	85,833	55,028	942	247,739
SRHA	39,724	11,157	22,391	9,081	10,154	66,599	36,847	337	196,290
WRHA	66,487	15,877	42,786	14,585	14,281	103,273	62,195	2,000	321,484
Grand Total	175,999	41,772	158,906	41,465	29,740	274,679	172,282	4,176	899,019

Table 26 summarizes the distribution of lubricants across entities and populations with a total of 31,820 pieces distributed. WRHA accounted for the largest distribution of 19,056 followed by NERHA with 5,727 across all groups. Of the key populations, FSW had the highest distribution of 11,871. Distribution to transgender populations (TG) was notably low and should be addressed, considering their high risk status.

Table 26: Lubricant Distribution by Entity and Target Population

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	0	0	0	0	0	0	52	0	52
Children First	0	0	0	0	0	0	21	3	24
Children of Faith	124	51	27	175	173	523	0	0	1,073
JASL	24	0	0	2	4	60	116	0	206
NERHA	535	118	162	703	490	3,424	261	34	5,727
SERHA	403	40	80	539	402	792	61	64	2,381
SRHA	281	49	41	541	274	1,654	422	39	3,301
WRHA	2,966	855	609	4,814	2,420	5,418	1,607	367	19,056
Grand Total	4,333	1,113	919	6,774	3,763	11,871	2,540	507	31,820

REFERRALS FOR SERVICES

Pre-Exposure Prophylaxis

Table 27 indicates that the majority of high-risk persons referred for PrEP are concentrated in SERHA and WRHA, with totals of 512 and 210, respectively. Other entities, such as NERHA, SRHA, ASHE, and Children First, have low referral numbers, suggesting lower overall risk or reporting. The data

implies the outreach team should engage clients in meaningful risk reduction conversations to identify high risk and document these engagements.

Table 27: Number Referred to and Initiated on PrEP Services by Entity and Population

Entity		ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	Referred	1	0	0	0	78	0	0	1	80
	Initiated	1	0	0	0	73	0	0	1	75
Children First	Referred	0	0	0	0	9	0	0	0	9
	Initiated	0	0	0	0	2	0	0	0	2
JASL	Referred	3	1	106	0	146	4	9	6	275
	Initiated	4	1	107	0	155	4	13	6	290
Entity		ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
NERHA	Referred	15	1	5	0	10	4	90	0	125
	Initiated	16	0	1	0	8	3	88	1	117
SERHA	Referred	156	9	33	9	25	117	160	3	512
	Initiated	39	1	0	0	12	2	67	0	121
SRHA	Referred	10	2	9	1	13	31	7	1	74
	Initiated	0	1	0	0	0	1	0	0	2
WRHA	Referred	37	11	48	3	19	54	35	3	210
	Initiated	0	0	0	0	0	1	1	0	2
Total	Referred	222	24	201	13	300	210	301	14	1,285
Total	Initiated	60	3	108	0	250	11	169	8	609

Psychological Services

Referrals to psychological support to promote positive behaviour change and reduce vulnerability is a key component of the HIV Prevention response. Table 28 indicates a very low number of high-risk person referrals across all entities, with only 76 individuals accessing psychological services. Key populations MSM (31) and FSW (31) accounted for the majority.

This data underscores a potential need for increased awareness and training among outreach teams to better identify and refer high-risk populations.

Table 28: Number Referred to Psychological Services by Entity and Populations

Entity	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	0	0	0	0	11	0	0	0	11
Children of Faith	2	0	1	0	0	0	1	0	4
JASL	0	0	26	0	17	2	0	2	47
NERHA	0	0	1	0	0	0	0	0	1
SERHA	2	0	1	0	1	1	0	0	5
SRHA	0	0	1	0	0	2	1	0	4
WRHA	0	0	1	1	2	0	0	0	4
Grand Total	4	0	31	1	31	5	2	2	76

Social Services

Access to social services is also included as part of the prevention package of care. These services involve interactions with medical social workers and case managers, who assist clients in accessing essential resources to mitigate social vulnerabilities. For the reporting period, only 14 cases were recorded in the DHIS2 system. This figure likely underrepresents the actual number of cases; the field will be engaged to improve reporting accuracy. Please see Table 29 for a detailed breakdown.

Table 29: Number Referred to Social Services by Entity and Populations

Entity	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	0	0	0	0	0	0	0	0	0
Children First	0	0	0	0	0	0	0	0	0
Children of Faith	1	0	0	0	0	0	0	0	1
JASL	0	0	6	0	2	0	0	0	8
NERHA	0	0	0	0	1	0	0	0	1
SERHA	0	1	0	0	0	0	0	0	1
SRHA	0	0	0	0	0	2	0	0	2
WRHA	0	0	1	0	0	0	0	0	1
Grand Total	1	1	7	0	3	2	0	0	14

Sexually Transmitted Infections Services

Sexually transmitted infections (STI) increase the risk of HIV acquisition. DHIS2 captures data on syphilis screening in outreach settings. Referral to treatment is a very important element of HIV Prevention services. The data in table 30 indicates that WRHA (508), SERHA (292), SRHA (280) and JASL (280) have the highest total cases, indicating larger populations or more active reporting in these regions. Referrals for key populations were as follows: MSM 373, FSW 224 and TG (35). For the other populations F50+ was had the highest which was 307. The data highlights the importance of timely referral to treatment and improved reporting to better address STI-related HIV risks across all entities.

Table 30: Number referred for Sexually Transmitted Infections Services by Entity and Populations

Entity	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	1	0	0	0	30	0	0	0	31
Children First	0	1	20	0	81	0	0	9	111
Children of Faith	3	0	0	0	0	1	1	0	5
JASL	10	4	68	3	174	8	7	6	280
NERHA	15	64	19	36	3	22	16	3	178
SERHA	42	47	34	36	20	64	36	13	292
SRHA	31	73	21	31	20	70	32	2	280
WRHA	45	118	62	78	45	93	65	2	508
Grand Total	147	307	224	184	373	258	157	35	1,685

PRE EXPOSURE PROPHYLAXIS (PrEP) SERVICES

In table 31 below, the data show a total of 135,759 individuals tested HIV negative, of whom 60,711 (45%) were engaged for PrEP as a viable prevention method. Only 1,285 (2%) of those engaged were referred. Of those referred, the majority were linked to PrEP services 677(53%) and 609 (90%) of these were successfully initiated on PrEP. The MSM, FSW and SAF populations exhibited the highest numbers in referral and linkage efforts, while groups such as TG and M50+ have very low participation. Overall, the high linkage to initiation rate suggests effective transition from linkage to PrEP initiation. There is a need to improve engagement across all groups.

Table 31: PrEP Uptake by Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	30,597	14,432	222	67	60
F50+	8,738	3,855	24	7	3
FSW	10,657	4,356	201	124	108
M50+	7,369	3,153	13	6	0
MSM	5,357	1,764	300	259	250
SAF	46,028	21,254	210	30	11
SAM	26,603	11,650	301	177	169
TG	410	247	14	7	8
Total	135,759	60,711 (45%)	1,285 (2%)	677 (53%)	609 (90%)

PREP UPTAKE BY ENTITY

ASHE had a total of 464 clients who tested negative for HIV, with 166 (36%) engaged about PrEP. Of those, 79 (48%) were referred. Subsequently, 73(92%) were linked and 74 (101%) were initiated. The initiation being greater than the linkage is due to the inconsistent update of the linkage field in the database. This is being addressed through engagement with the field. Table 32 contains a detailed breakdown.

Table 32: ASHE PrEP by Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
MSM	460	165	78	72	73
TG	4	1	1	1	1
ARY	50	0	1	1	1
SAF	2	0	0	0	0
SAM	6	0	0	0	0
M50+	2	0	0	0	0
Grand Total	464	166 (36%)	80 (48%)	74 (93%)	75 (101%)

Children First had a total of 1,301 clients who tested negative for HIV, with 216 (17%) engaged about PrEP. Of those, 9 (4%) were referred. Subsequently, 3(50%) were linked and none was initiated. The main population engaged were MSM (188), followed by FSW (20).

Table 33: Children First PrEP Uptake by Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	1	0	0	0	0
F50+	3	0	0	0	0
FSW	691	20	0	0	0
M50+	0	0	0	0	0
MSM	579	188	9	3	0
SAF	3	0	0	0	0
SAM	4	0	0	0	0
TG	20	8	0	0	0
Grand Total	1,301	216 (17%)	9 (4%)	3 (33%)	0 (0%)

JASL had a total of 6,383 clients who tested negative for HIV, with 785 (12%) engaged about PrEP. Of those, 275 (35%) were referred. Subsequently, 288 (105%) were linked and 290 (101%) were initiated. There are discrepancies with the number of client referred, linked and initiated on PrEP. This is attributable to the inconsistent updating of these fields in DHIS 2.

Table 34: JASL PrEP Uptake for July to September 2025 By Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	899	122	3	4	4
F50+	119	4	1	1	1
FSW	1,774	266	106	107	107
M50+	140	10	0	0	0
MSM	1,610	208	146	154	155
SAF	1,151	127	4	4	4
SAM	666	47	9	12	13
TG	24	1	6	6	6
Grand Total	6,383	785 (12%)	275 (35%)	288 (105%)	290 (101%)

NERHA had a total of 16,704 clients who tested negative for HIV, with 5,724 (34%) engaged about PrEP. Of those, 125 (2%) were referred. Subsequently, 115 (92%) were linked and 117 (102%) were initiated. The number of clients referred and linked are greater than those initiated. This is attributable to the inconsistent updating of these fields in DHIS 2.

Table 35: NERHA PrEP Uptake for July to September 2025 By Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	3,201	1,141	15	14	16
F50+	1,674	855	1	0	0
FSW	1,056	306	5	1	1
M50+	1,200	519	0	0	0
MSM	244	48	10	8	8
SAF	5,716	1,931	4	3	3
SAM	3,581	915	90	89	88
TG	32	9	0	0	1
Grand Total	16,704	5,724 (34%)	125 (2%)	115 (92%)	117 (102%)

SERHA had a total of 44,457 clients who tested negative for HIV, with 29,510 (66%) engaged about PrEP. Of those, 512 (2%) were referred. Subsequently, 159 (31%) were linked and 121 (76%) initiated. Sexually active males accounted for the highest initiations (67) followed by ARY with 39.

Table 36: SERHA PrEP Uptake for January to December 2025 By Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	11,954	7,725	156	44	39
F50+	2,090	1,347	9	6	1
FSW	1,907	1,540	33	5	0
M50+	2,141	1,335	9	3	0
MSM	294	255	25	13	12
SAF	16,188	10,886	117	18	2
SAM	9,771	6,321	160	70	67
TG	112	101	3	0	0
Grand Total	44,457	29,510 (66%)	512 (2%)	159 (31%)	121 (76%)

SRHA had a total of 27,774 clients who tested negative for HIV, with 5,247(18%) engaged about PrEP. Of those, 74 (1%) were referred, 1 (1%) linked and 2 (200%) were initiated. The number of clients initiated are greater than those linked. This is attributable to the inconsistent updating of these fields in DHIS 2.

Table 37: SRHA PrEP Uptake for January to December 2025 By Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	5,981	1,220	10	1	0
F50+	2,344	402	2	0	1
FSW	1,484	288	9	0	0
M50+	1,711	284	1	0	0
MSM	837	229	13	0	0
SAF	10,067	1,869	31	0	1
SAM	5,321	949	7	0	0
TG	29	6	1	0	0
Grand Total	27,774	5,247 (18%)	74 (1%)	1 (1%)	2 (200%)

WRHA had a total of 38,355 clients who tested negative for HIV, with 19,063 (50%) engaged about PrEP. Of those, 210 (1%) were referred. Subsequently, 36 (17%) were linked and 2 (6%) initiated. Only SAM and SAF had one initiation each.

Table 38: WRHA PrEP Uptake for January to December 2025 By Targeted Populations

Population	HIV Negative	Engaged	Referred	Linked	Initiated
ARY	8,473	4,223	37	3	0
F50+	2,490	1,247	11	0	0
FSW	3,707	1,936	48	10	0
M50+	2,158	1,005	3	3	0
MSM	1,329	671	19	9	0
SAF	12,810	6,441	54	5	1
SAM	7,199	3,418	35	6	1
TG	189	122	3	0	0
Grand Total	38,355	19,063 (50%)	210 (1%)	36 (17%)	2 (6%)

The Prevention Programme provides two methods for PrEP administration: daily oral and event-driven. However, data entry discrepancies are affecting the accuracy and consistency of recording which method is being used. Table 39 presents a snapshot of the current data breakdown, highlighting these discrepancies.

Table 39: PrEP Methods for January to December 2025 By Targeted Populations

Entity	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Grand Total
ASHE	1	0	0	0	0	0	72	1	74
Daily Oral	1	0	0	0	0	0	70	1	72
Event Driven	0	0	0	0	0	0	2	0	2
Children First	0	0	0	0	0	0	2	0	2
Daily Oral	0	0	0	0	0	0	2	0	2
JASL	4	1	0	4	12	107	149	6	283
Daily Oral	4	1	0	4	12	107	149	6	283
NERHA	13	0	0	3	87	1	7	0	111
Daily Oral	1	0	0	2	2	1	1	0	7
SERHA	39	1	0	2	67	0	12	0	121
Daily Oral	1	1	0	1	1	0	0	0	4
Event Driven	38	0	0	1	66	0	12	0	117
WRHA	0	0	0	0	1	0	0	0	1
Daily Oral	0	0	0	0	1	0	0	0	1
Grand Total	57	2	0	9	167	108	242	7	592

REFERRALS FOR HIV DIAGNOSIS

Referral to HIV services is a crucial component of the HIV Prevention programme. Clients who test preliminarily positive for HIV need to be referred for confirmatory testing and appropriate care. Table 40, shows the number of clients reporting high-risk behaviours who tested preliminary positive. The highest numbers were observed in the MSM (22) and FSW (18) populations.

Table 40: Number of Clients Reporting High-risk Behaviours with Positive HIV Screening by Entity and Populations from January to December 2025

Entity	ARY	F50+	FSW	M50+	MSM	SAF	SAM	TG	Grand Total
ASHE	0	0	0	0	1	0	0	0	1
Children First	0	0	2	0	1	0	0	1	4
Children of Faith	1	0	0	0	0	0	0	0	1
JASL	0	0	2	0	5	0	1	0	8
NERHA	0	1	4	0	1	2	2	0	10
SERHA	2	1	3	3	6	4	2	0	21
SRHA	1	0	2	0	0	2	2	0	7
WRHA	2	1	5	0	8	3	5	0	24
Grand Total	6	3	18	3	22	11	12	1	76

LINKAGE TO HIV TREATMENT AND CARE

Table 41 indicates that 526 individuals tested preliminary positive for HIV across various populations, of which 103 cases were confirmed positive, and 87 were linked to care. The SAF population had the highest number of preliminary positives (135), but a relatively low confirmation rate of 16% (22), with 18 linked to care. The MSM showed a confirmation rate of 25% (24 out of 95) and most were linked to care (21). Overall, while a substantial number of clients are identified as preliminary positives, there is room to improve documentation of linkage to care in DHIS2. The linking of DHIS2 to the TSIS database will help to address this issue. In the interim, manual comparisons of both databases are conducted to confirming the linkage to care numbers.

Table 41: Number of HIV Preliminary Positives, Confirmed Positives and Linked to Care

Population	Preliminary HIV Positive	Confirmed Positive	Linked to Care
ARY	53	12	11
F50+	54	13	12
FSW	56	8	6
M50+	42	12	8
MSM	95	24	21
SAF	135	22	18
SAM	80	12	11
TG	11	0	0
Grand Total	526	103	87

DATA QUALITY ACTIVITIES JANUARY- DECEMBER 2025

The 2025 National Prevention Data Quality Assessment Audit was conducted between March and April, in the four Regional Health Authorities and one Civil Society Organization (CSO), Jamaica Aids Support for Life (JASL). This audit focused on ten indicators that are essential for generating the Unique Identify Code (UIC) as well as for gathering outreach and demographic data. Furthermore, the assessment included evaluations of the organization, data collection methods, report preparation, and storage, conducted through interviews with the site personnel.

The national target for the 2025 DQA score was established at 95%, which facilitates a 5% margin of error. The overall performance for 2025 resulted in no region achieving the target. The achievements are as follows: NERHA 88%, SERHA 87%, JASL 85%, SRHA 83%, and WRHA 73%. The 2025 audit was limited to the Key Populations; however, for 2026, the audit will cover all populations.

Table 42: National Prevention Data Quality Assessment Audit Scoring for 2025

Entity	National Target	2025 Achievement	Ranking
NERHA	95%	88%	Good
SERHA	95%	87%	Good
JASL	95%	85%	Good
SRHA	95%	83%	Needs Improvement
WRHA	95%	73%	Needs improvement

Common errors identified on a monthly basis are incorrect event date (e.g. dates that are in the future), late report that affects previous reports, as well as ownership of entries in the deduplication process. For PrEP, there are incomplete entries, for example, missing documentation of referrals, linkage or initiation dates. Monthly data quality checks are carried out as part of the Payment for Results process under the Prevention, programme followed by an annual audit conducted by the Strategic Information Component.

ACTION PLAN

Based on the challenges and limitations outlined in the report, the following actions are recommended:

Action Items	Responsible Officer	Time Line
Enable individual-level tracking by installing DHIS2 "Tracker Programme" functionality, which may require a significant restructuring of the current database.	Strategic Information, HSTU	Ongoing
Conduct data quality audit activities to address inconsistent updating of entries in DHIS2 e.g. referral and linkage for PrEP fields, condom distribution	BCC Coordinators	Within the quarter
Conduct monthly comparison of confirmed HIV positive cases in DHIS2 to newly HIV confirmed confirmed/linked cases in TSIS	Strategic Information Component, HSTU	Ongoing
Engage CSOs about lack of entry in DHIS2 for HIV self-testing	Prevention Component, HSTU	Ongoing
Provide list of unlinked patients to the respective Contact Investigators to facilitate entry into care	BCC Coordinators	Within the quarter